

LCD Reference Article

Billing and Coding Article

Billing and Coding: Wound Application of Cellular and/or Tissue Based Products (CTPs), Lower Extremities

A56696

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Contractor Information

Contractor Name	Contract Type	Contract Number	Jurisdiction	States
CGS Administrators, LLC	MAC - Part A	15101 - MAC A	J - 15	Kentucky
CGS Administrators, LLC	MAC - Part B	15102 - MAC B	J - 15	Kentucky
CGS Administrators, LLC	MAC - Part A	15201 - MAC A	J - 15	Ohio
CGS Administrators, LLC	MAC - Part B	15202 - MAC B	J - 15	Ohio

Article Information

General Information

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Article Title

Billing and Coding: Wound Application of Cellular and/or Tissue Based Products (CTPs), Lower Extremities

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CMS National Coverage Policy

Relevant CMS manual instructions and policies regarding bioengineered skin substitutes are found in the following Internet-Only Manuals (IOMs) published on the CMS Web site:

- CMS Internet-Only Manual (IOM), Pub. 100-04, Medicare Claims Processing Manual, Chapter 17, Section 40.
- CMS Internet-Only Manual (IOM). Pub. 100-03, Medicare National Coverage Determinations Manual, Chapter 1, Part 4, Section 270.1.
- CMS Change Request, CR 8213; Autologous Platelet-Rich Plasma (PRP) for Chronic Non-Healing Wounds; issued June 10, 2013
- CY 2026 Payment Policies Under the Physician Fee Schedule and Other Changes to Part B Payment and Coverage Policies; Medicare Shared Savings Program Requirements; and Medicare Prescription Drug Inflation Rebate Program, CMS-1832-F, 90 Fed. Reg. 49266 (Nov. 5, 2025)

Social Security Act (XVIII) Standard References:

- Title XVIII of the Social Security Act, 1862(a)(1)(A) states that no Medicare payment shall be made for items or services which are not reasonable and necessary for the diagnosis or treatment of illness or injury.
- Title XVIII of the Social Security Act, Section 1833(e) states that no payment shall be made to any provider for any claim that lacks the necessary information to process the claim.
- Title XVIII of the Social Security Act, Section 1862(a)(7). This section excludes routine physical examinations.

Article Guidance

Article Text

This article gives guidance for billing, coding, and other guidelines in relation to local coverage policy Wound Application of Cellular and/or Tissue Based Products (CTPs), Lower Extremities L36690.

Coding Guidance

We are no longer publishing a list of HCPCS codes because it is not necessary to adjudicate the claim.

Do not report non-graft wound dressings or injected skin substitute HCPCS codes with skin substitute graft/cellular and/or tissue-based products (CTP) and HCPCS application codes as this would be considered incorrect coding. Such products are bundled into other standard management procedures if medically necessary and are not separately payable.

Removal of a current graft and/or simple cleansing of the wound and other surgical preparation services are included in the skin substitute graft/CTP and HCPCS application codes. Active wound care management (CPT code 97602. Removal of devitalized tissue from wound(s), non-selective debridement, without anesthesia (e.g., wet-to-moist dressings, enzymatic, abrasion, larval therapy), including topical application(s), wound assessment, and instruction(s) for ongoing care, per session procedures should never be reported in conjunction with skin substitute graft/CTP and HCPCS application codes.

An evaluation and management (E/M) service should only be reported with a skin replacement therapy (application of skin substitute graft/CTP) if the patient required a service that was separate and distinct from the skin replacement service.

Do not report 15271–15278 when a skin substitute is used for anything other than skin replacement.

If reporting a skin substitute product with HCPCS code A4100 (Skin substitute, FDA cleared as a device, not otherwise specified code, Q4431 (Unlisted PMA skin substitute product), Q4432 (Unlisted 501k skin substitute product) or Q4433 (Unlisted 361 HCT/P skin substitute product), the product name, package size purchased, amount applied and amount wasted must be reported in the claim narrative/remarks or the claim will be returned to the provider/rejected.

Part A:

Line Level Remarks for 837I Electronic Claim: 837I 2400 SV202-7

Claim Level Remarks for 837I Electronic Claim: 837I 2300 NTE*ADD

Block 80 for the UB04 claim form

“REMARKS” field for a DDE claim

Part B:

Loop 2400 or SV101-7 for the 5010A1 837P

Box 19 for paper claim

- The name of the product, size, and the amount used must appear in the Documentation Field.

For codes that are not reimbursed at a standardized payment per the CY 2026 Medicare Physician Fee Schedule (PFS) Final Rule if the charge matches the actual invoice cost, note "Actual Invoice Cost" in the Documentation Field. You are not required to submit invoice information with the claim; however, it must be available if requested.

- If you are submitting a charge greater than the actual invoice cost, please include the following information in the Documentation Field, using these abbreviations:
 - Des = Description/Name of skin sub/product ()
 - QS = Quantity shipped (e.g., QS=3 boxes)
 - TA = Total amount charged for quantity shipped (e.g., TA=\$437.50)
 - UP = Unit Price (e.g., UP = \$17.50 per 5X5 cm) (Optional)
 - DG = Amount used (e.g., 25 cm)

The appropriate CPT or HCPCS application code must be reported on the same claim as the skin substitute graft/CTP HCPCS code. The claim will be returned to provider or denied if the application code and skin substitute graft/CTP code are not submitted on the same claim. When the skin substitute graft/CTP HCPCS code is denied, the related application code will also be denied.

CGS Administrators expects that where multiple sizes of a specific product are available, the size that best fits the wound with the least amount of wastage will be utilized. Each product

material is for a single-use only and is not for use on a multiple patients.

Advance Beneficiary Notice of Non-coverage (ABN) Modifier Guidelines

It is not appropriate to bill Medicare for services that are not covered as if they are covered. When billing for non-covered services, use the appropriate modifier. For Part A and Part B use the use GY (Item or service statutorily excluded, does not meet the definition of any Medicare benefit or, for non-Medicare insurers, is not a contract benefit) or GZ (Item or service expected to be denied as not reasonable and necessary) modifier.

An ABN may be used for services which are likely to be non-covered, whether for medical necessity or for other reasons. Refer to CMS Publication 100-04, Medicare Claims Processing Manual, Chapter 30, for complete instructions.

Effective from April 1, 2010, non-covered services should be billed with modifier –GA, -GX, -GY, or –GZ, as appropriate.

The –GA modifier (“Waiver of Liability Statement Issued as Required by Payer Policy”) should be used when physicians, practitioners, or suppliers want to indicate that they anticipate that Medicare will deny a specific service as not reasonable and necessary and they do have an ABN signed by the beneficiary on file. Modifier GA applies only when services will be denied under reasonable and necessary provisions, sections 1862(a)(1), 1862(a)(9), 1879(e), or 1879(g) of the Social Security Act. Effective April 1, 2010, Part A MAC systems will automatically deny services billed with modifier GA. An ABN, Form CMS-R-131, should be signed by the beneficiary to indicate that he/she accepts responsibility for payment. The -GA modifier may also be used on assigned claims when a patient refuses to sign the ABN and the latter is properly witnessed. For claims submitted to the Part A MAC, occurrence code 32 and the date of the ABN is required.

Modifier GX (“Notice of Liability Issued, Voluntary Under Payer Policy”) should be used when the beneficiary has signed an ABN, and a denial is anticipated based on provisions other than medical necessity, such as statutory exclusions of coverage or technical issues. An ABN is not required for these denials, but if non-covered services are reported with modifier GX, will automatically be denied services.

The –GZ modifier should be used when physicians, practitioners, or suppliers want to indicate that they expect that Medicare will deny an item or service as not reasonable and necessary and they have not had an ABN signed by the beneficiary. If the service is statutorily non-covered, or without a benefit category, submit the appropriate CPT/HCPCS code with the -GY modifier. An ABN is not required for these denials, and the limitation of liability does not apply for beneficiaries. Services with modifier GY will automatically deny.

Documentation Requirements

1. All documentation must be maintained in the patient's medical record and made available to the contractor upon request.

2. Every page of the record must be legible and include appropriate patient identification information (e.g., complete name, dates of service(s)). The documentation must include the legible signature of the physician or non-physician practitioner responsible for and providing the care to the patient.
3. The submitted medical record must support the use of the selected ICD-10-CM code(s). The submitted CPT/HCPCS code must describe the service performed.
4. Medical record documentation must support the medical necessity of the services as directed in this policy.
5. The documentation must support that the service was performed and must be included in the patient's medical record. This information is normally found in the history and physical, office/progress notes, hospital notes, and/or procedure report.
6. The medical record must clearly show that the criteria listed under the "Indications and Limitations of Coverage and/or Medical Necessity" sections have been met, as well as, the appropriate diagnosis and response to treatment.
7. The documentation must support the need for CTP application and the product used.
8. A description of the wound(s) must be documented at baseline (prior to beginning conservative treatment) relative to size, location, stage, duration, and presence of infection, in addition to type of treatment given and response.
 - a. This information must be updated in the medical record throughout treatment.
 - b. Wound description must also be documented pre and post treatment with the skin substitute graft being used.
 - c. If obvious signs of worsening or lack of treatment response is noted, continuing treatment with the skin substitute would not be considered medically reasonable and necessary without documentation of a reasonable rationale for doing so.
9. Documentation of smoking history, and that the patient has received counseling on the effects of smoking on surgical outcomes and treatment for smoking cessation (if applicable) as well as outcome of counselling must be in the medical record.
10. An operative note must support the procedure (e.g., application of skin substitute graft/CTPs to legs) for the relevant date of service (first application starts the 12 week episode of care) and include the reason for the procedure and a complete description of the procedure including product used (with identifying package label in the chart), and relevant findings.
11. The HCPCS code of the applicable skin substitute graft/CTP and the units billed must be consistent with the medical record regarding wound description and size.

Utilization Guidelines

- A maximum of 10 skin substitute graft/CTP applications per ulcer will be allowed for the episode of skin replacement surgery (defined as 12 weeks from the first application of a skin substitute graft/CTP). Product change within the episode of skin replacement

surgery may be appropriate. When more than 1 specific product is used during the 12-week period, it is expected that the total number of applications or treatments will still not exceed 10.

- Greater than ten (10) applications for the treatment of a single wound within a 12-week period of time, will be considered Not Reasonable and Necessary.
- Separately billed repeated use of the skin substitute after 12 weeks for a single wound or episode is non-covered.
- The utilization of a CTP non-compliant with medical necessity or designated guidelines for that specific product may necessitate review or non-coverage as not medically necessary.
- Labeling for most skin substitute grafts include language suggesting multiple applications; however, Medicare does not expect that every ulcer in every patient will require the

maximum number of applications listed on the product label or allowed for reimbursement.

- Utilization rates that exceed peer norms, identified through data analysis may prompt prepayment or post payment medical review.

Multiple Wounds

- To determine the surface area for application of skin substitute graft code selection for multiple wounds, all areas within the same anatomic grouping should be added. If the surgeon applies skin grafts to wounds on a different anatomic area, they should bill the corresponding application code for legs or feet.
- Do not code modifier -59 on skin substitute, graft application or skin substitute product codes. Skin substitute graft application codes are appropriately coded based upon total surface area of anatomical locations and not by number of ulcers.
- Modifier -50 and modifiers -LT and -RT are not appropriately appended to skin substitute codes. Coding for skin substitute graft application is based upon total surface area of the ulcers; therefore, Modifiers -50, -LT and -RT are not required for proper claim adjudication.

Incident-to-Supply Classification

Effective January 1, 2026, most skin substitutes will no longer be treated as “biologics” and will not be reimbursed under the Average Sales Price (ASP) methodology. Instead, they will be classified as **incident-to supplies**, receiving a **flat-rate payment per sq cm**, regardless of brand or product type. This change applies across physician offices and outpatient settings to standardize reimbursement and reduce cost variability.

Wastage Policy

Under the incident-to supply classification:

- Medicare will not provide separate payment for discarded or unused portions of skin substitute products.
 - Only the amount **actually applied to the patient** will be eligible for reimbursement at the per-square-centimeter rate.
 - Providers must:
 - Accurately calculate the amount needed before opening a product.
 - Document the exact amount applied in the medical record.
 - Avoid over-ordering or unnecessary product wastage, as this will result in unreimbursed cost.

Providers are encouraged to adopt scheduling practices that minimize single-use or limited-shelf-life product waste and unnecessary resource use. Optimizing scheduling not only improves clinical efficiency but also supports sustainable, cost-effective healthcare delivery.

Coding Information

CPT/HCPCS Codes

Please accept the License to see the codes.

CPT/HCPCS Modifiers

N/A

ICD-10-CM Codes that Support Medical Necessity

Group 1 (41 Codes)

Group 1 Paragraph

We are no longer publishing a list of HCPCS codes because it is not necessary for adjudicating the claim.

The following ICD-10 codes align with the coverage criteria in the LCD. Select the most appropriate and specific code that aligns with the care provided.

Group 1 Codes

Code	Description
E08.621	Diabetes mellitus due to underlying condition with foot ulcer
E09.621	Drug or chemical induced diabetes mellitus with foot ulcer
E10.621	Type 1 diabetes mellitus with foot ulcer
E11.621	Type 2 diabetes mellitus with foot ulcer
E13.621	Other specified diabetes mellitus with foot ulcer
I83.011	Varicose veins of right lower extremity with ulcer of thigh
I83.012	Varicose veins of right lower extremity with ulcer of calf
I83.013	Varicose veins of right lower extremity with ulcer of ankle
I83.014	Varicose veins of right lower extremity with ulcer of heel and midfoot
I83.015	Varicose veins of right lower extremity with ulcer other part of foot
I83.018	Varicose veins of right lower extremity with ulcer other part of lower leg
I83.021	Varicose veins of left lower extremity with ulcer of thigh
I83.022	Varicose veins of left lower extremity with ulcer of calf
I83.023	Varicose veins of left lower extremity with ulcer of ankle
I83.024	Varicose veins of left lower extremity with ulcer of heel and midfoot

Code	Description
I83.025	Varicose veins of left lower extremity with ulcer other part of foot
I83.028	Varicose veins of left lower extremity with ulcer other part of lower leg
I83.211	Varicose veins of right lower extremity with both ulcer of thigh and inflammation
I83.212	Varicose veins of right lower extremity with both ulcer of calf and inflammation
I83.213	Varicose veins of right lower extremity with both ulcer of ankle and inflammation
I83.214	Varicose veins of right lower extremity with both ulcer of heel and midfoot and inflammation
I83.215	Varicose veins of right lower extremity with both ulcer other part of foot and inflammation
I83.218	Varicose veins of right lower extremity with both ulcer of other part of lower extremity and inflammation
I83.221	Varicose veins of left lower extremity with both ulcer of thigh and inflammation
I83.222	Varicose veins of left lower extremity with both ulcer of calf and inflammation
I83.223	Varicose veins of left lower extremity with both ulcer of ankle and inflammation
I83.224	Varicose veins of left lower extremity with both ulcer of heel and midfoot and inflammation

Code	Description
I83.225	Varicose veins of left lower extremity with both ulcer other part of foot and inflammation
I83.228	Varicose veins of left lower extremity with both ulcer of other part of lower extremity and inflammation
I87.011	Postthrombotic syndrome with ulcer of right lower extremity
I87.012	Postthrombotic syndrome with ulcer of left lower extremity
I87.013	Postthrombotic syndrome with ulcer of bilateral lower extremity
I87.031	Postthrombotic syndrome with ulcer and inflammation of right lower extremity
I87.032	Postthrombotic syndrome with ulcer and inflammation of left lower extremity
I87.033	Postthrombotic syndrome with ulcer and inflammation of bilateral lower extremity
I87.311	Chronic venous hypertension (idiopathic) with ulcer of right lower extremity
I87.312	Chronic venous hypertension (idiopathic) with ulcer of left lower extremity
I87.313	Chronic venous hypertension (idiopathic) with ulcer of bilateral lower extremity
I87.331	Chronic venous hypertension (idiopathic) with ulcer and inflammation of right lower extremity

Code	Description
187.332	Chronic venous hypertension (idiopathic) with ulcer and inflammation of left lower extremity
187.333	Chronic venous hypertension (idiopathic) with ulcer and inflammation of bilateral lower extremity

ICD-10-CM Codes that DO NOT Support Medical Necessity

N/A

ICD-10-PCS Codes

N/A

Additional ICD-10 Information

N/A

Bill Type Codes

Contractors may specify Bill Types to help providers identify those Bill Types typically used to report this service. Absence of a Bill Type does not guarantee that the article does not apply to that Bill Type. Complete absence of all Bill Types indicates that coverage is not influenced by Bill Type and the article should be assumed to apply equally to all claims.

Code	Description
Please accept the License to see the codes.	

Revenue Codes

Contractors may specify Revenue Codes to help providers identify those Revenue Codes typically used to report this service. In most instances Revenue Codes are purely advisory. Unless specified in the article, services reported under other Revenue Codes are equally subject to this coverage determination. Complete absence of all Revenue Codes indicates that coverage is not influenced by Revenue Code and the article should be assumed to apply equally to all Revenue Codes.

Not Applicable

Code	Description
Please accept the License to see the codes.	

Other Coding Information

N/A

Revision History Information

Revision History Date	Revision History Number	Revision History Explanation
04/26/2026	R25	Revision Effective: 03/12/2026 Revision Explanation: Update to article texts and covered codes.
09/04/2025	R24	Revision Effective: 09/04/2025 Revision Explanation: Annual review, no changes.
09/05/2024	R23	Revision Effective: 09/05/2024 Revision Explanation: Correction of invalid NCD.
09/05/2024	R22	Revision Effective: 09/05/2024 Revision Explanation: Annual review, no changes.
08/01/2024	R21	Revision Effective: 08/01/2024 Revision Explanation: Q4304 is being added retro effective 01/01/2024 to group 2 CPT/HCPCS code section.
02/29/2024	R20	Revision Effective: 02/29/2024 Revision Explanation: A2009 has been added to group 2 HCPCS codes and is retro effective back to 10/01/2023.
12/28/2023	R19	Revision Effective: 12/28/2023

Revision History Date	Revision History Number	Revision History Explanation
		Revision Explanation: Added Q4236 to group 2 HCPCS codes, retro effective to 01/01/2023.
12/28/2023	R18	Revision Effective: 12/28/2023 Revision Explanation: added Q4278 to group 2 HCPCS/CPT and is retro effective back to 07/01/2023.
11/16/2023	R17	Revision Effective: 11/16/2023 Revision Explanation: Updated LCD Reference Article section.
01/01/2023	R16	Revision Effective: 01/01/2023 Revision Explanation: Added A2002 and Q4262 to group 2 HCPCS/CPT code list. A2002 is retro effective to 01/01/2022 and Q4262 is retro effective to 01/01/2023.
09/01/2022	R15	Revision Effective: 09/01/2022 Revision Explanation: Annual review, no changes.
04/28/2022	R14	Revision Effective: 04/28/2022 Revision Explanation: Added A2008 to group 2 in the CPT/HCPCS section.
04/14/2022	R13	Revision Effective: 04/14/2022

Revision History Date	Revision History Number	Revision History Explanation
		Revision Explanation: Added Q4253 to Group 2 codes and this is effective retroactive 10/01/2021.
03/25/2022	R12	Revision Effective: 03/25/2022 Revision Explanation: Moved A2001 from group 1 to group 2 as it was added to the incorrect group. Added new product A2004 to group 2.
03/03/2022	R11	Revision Effective: 02/17/2022 Revision Explanation: HCPCS code Q4174 and Q4231 were removed from CPT/HCPCS codes group 2 as they are not FDA approved for the indications outlined in policy L36690. Added A2001 to Group 1 codes.
09/09/2021	R10	Revision Effective: 09/01/2021 Revision Explanation: Added new code Q4254 to group 2 HCPCS/CPT codes.
09/09/2021	R9	Revision Effective: 09/09/2021 Revision Explanation: HCPCS code Q4145 and Q4177 were removed from CPT/HCPCS codes group 2 as they are not FDA approved for the indications outlined in policy L36690.

Revision History Date	Revision History Number	Revision History Explanation
08/19/2021	R8	Revision Effective: 08/19/2021 Revision Explanation: Annual review, no changes were made.
03/25/2021	R7	Revision Effective: 03/25/2021 Revision Explanation: Added Q4180, Q4231, and Q4232 to group 2 CPT HCPCS group.
10/08/2020	R6	Revision Effective: 10/08/2020 Revision Explanation: Added Q4201 to Group 2
10/01/2020	R5	Revision Effective: 10/01/2020 Revision Explanation: Added Q4176 and Q4170 to A56696 Group 2
07/01/2020	R4	Revision Effective: n/a Revision Explanation: Annual review, no changes made
07/01/2020	R3	R3 Revision Effective; 07/01/2020 Revision Explanation: Added new codes Q4237-Q4239 that were effective 07/01/2020 in the paragraph for group 2 as they have not been loaded to the database yet.

Revision History Date	Revision History Number	Revision History Explanation
10/31/2019	R2	R2 Revision Effective; 10/31/2019 Revision Explanation: Updated the documentation requirements information and added utilization guidelines that was listed in associated information from the policy.
09/19/2019	R1	Revision Effective: 09/19/2019 Revision Explanation: Converted article into new Billing and Coding template no other changes made.

Associated Documents

Related Local Coverage Documents

LCDs

[L36690 - Wound Application of Cellular and/or Tissue Based Products \(CTPs\), Lower Extremities](#) 

Related National Coverage Documents

NCDs

[270.1 - Electrical Stimulation \(ES\) and Electromagnetic Therapy for the Treatment of Wounds](#) 

Statutory Requirements URLs

N/A

Rules and Regulations URLs

N/A

CMS Manual Explanations URLs

N/A

Other URLs

N/A

Public Versions

Updated On	Effective Dates	Status	
03/05/2026	04/26/2026 - N/A	Currently in Effect	You are here
08/29/2025	09/04/2025 - 04/25/2026	Superseded	View
04/18/2025	09/05/2024 - 09/03/2025	Superseded	View
08/26/2024	09/05/2024 - N/A	Superseded	View

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Keywords

N/A